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Centers for Medicare & Medicaid Services
U.S. Department of Health and Human Services
200 Independence Avenue SW
Washington, DC 20201

Subject: Comments on the Federal Register Request for Information – Advancing a Patient-Centered Digital Health Infrastructure

To Whom It May Concern:
I appreciate the opportunity to submit comments in response to the Request for Information (RFI) issued by the Centers for Medicare & Medicaid Services (CMS) in partnership with the Assistant Secretary for Technology Policy/Office of the National Coordinator for Health Information Technology (ASTP/ONC). As Director of Information Technology for the South Carolina Primary Health Care Association and Program Director for the South Carolina Health Center Controlled Network (SCHCCN), I am committed to advancing equitable, secure, and patient-centered digital health innovation for Community Health Centers and the underserved populations they serve.

Please find below my comments aligned with the RFI focus areas:

Patients & Caregivers
CMS should collaborate with digital literacy initiatives to ensure accessibility of digital tools for seniors, low-income individuals, and marginalized populations. I recommend the funding of pilot programs aimed at developing "navigation-first" applications that support appointment management, care team communication, and patient-driven health goal tracking.

Providers
Provider burden related to quality reporting and interoperability compliance continues to be a significant challenge. CMS should consider enabling EHR vendors to transmit aggregated, risk-adjusted quality metrics via Bulk FHIR APIs. This would help reduce manual reporting, enhance data accuracy, and alleviate workflow inefficiencies.

Payers
Many payers currently lack standardized APIs to support seamless data exchange. CMS should align payer requirements with Blue Button and Data at Point of Care standards and tie continued participation in CMS programs to measurable progress in digital identity adoption and FHIR API readiness.

Technology Vendors & Networks
ONC certification pathways remain difficult to navigate for developers, particularly those seeking to expand beyond traditional app use cases. I encourage CMS to establish a "Digital Health Innovation Certification Track" that emphasizes real-world testing, usability, and interoperability over rigid modular certification requirements.

Value-Based Care Organizations
Value-based care models rely heavily on timely, actionable data. Unfortunately, current gaps in data transmission and longitudinal data integration hinder care coordination. CMS should incentivize technology vendors to natively support capabilities such as event notifications, patient matching, and care planning APIs in VBC-certified platforms.

Additional Recommendations
Establish a Centralized Digital Health Resource Hub: CMS should support the creation of a national hub for digital health developers, offering sandbox environments, interoperability toolkits, standards documentation, and patient-centered design feedback loops.

Adopt a Metrics-Driven Evaluation Framework: Implement digital maturity benchmarks to measure API adoption, patient access and app utilization rates, and improvements in care coordination.

Expand Blue Button 2.0 Beyond Medicare: Extend the Blue Button program to Medicaid and dual-eligible beneficiaries to strengthen access and accelerate innovation for underserved populations.

These recommendations reflect the realities of safety-net providers working to integrate modern technologies while ensuring equitable access, efficiency, and outcomes. I commend CMS and ASTP/ONC for advancing this important dialogue and look forward to future collaborations that promote a seamless, secure, and inclusive digital health ecosystem.

Sincerely,
Chandra Beasley